Emergency Department Triage and Patient Flow Procedure

Document ID CGH-ED-003. Version 1.0. Effective 1 January 2026. Owner: Head of Emergency Medicine.

1. Purpose

This procedure explains how patients move through the Emergency Department at CareOps General Hospital, how triage priorities are assigned, and how operational bottlenecks are recognised. It is an operations procedure and does not replace clinical judgement.

2. Triage system used

The hospital uses the Emergency Severity Index, known as ESI, a five-level triage algorithm developed in the United States and widely used in emergency departments. A trained triage nurse assigns the level. ESI level 1 is the highest priority and means the patient needs an immediate life-saving intervention. ESI level 2 means a high-risk situation, a confused or lethargic patient, or severe pain or distress, and the patient should not wait. ESI level 3 means the patient is stable but is expected to need two or more resources, such as laboratory tests, imaging or intravenous fluids. ESI level 4 means one expected resource. ESI level 5 means no resources are expected, for example a prescription refill or a simple wound check. Triage is a dynamic process and the level is changed if the patient condition changes while waiting.

3. Hospital door-to-doctor targets

CareOps General Hospital sets these internal targets for time from arrival to first doctor contact. ESI 1 is seen immediately. ESI 2 is seen within 10 minutes. ESI 3 is seen within 30 minutes. ESI 4 is seen within 60 minutes. ESI 5 is seen within 120 minutes. These targets are internal policy. For comparison, the Manchester Triage System, used in the United Kingdom and other countries, uses five colour categories with maximum target times of immediate for red, 10 minutes for orange, 60 minutes for yellow, 120 minutes for green and 240 minutes for blue.

4. Patient flow stages

Patient flow in the ED has these stages. First is arrival by walk-in, ambulance or transfer. Second is registration, which should be quick and can be completed after treatment starts in critical cases. Third is triage within 5 minutes of arrival. Fourth is placement in a resuscitation bay, treatment cubicle, fast track area or waiting area according to the ESI level. Fifth is the doctor assessment. Sixth is diagnostics such as laboratory tests and imaging. Seventh is treatment and reassessment. Eighth is the disposition decision, which is discharge, admission, transfer or observation. Ninth is the physical exit from the ED. The longest delays usually occur at the diagnostic stage and at the stage between the admission decision and the physical transfer to an inpatient bed.

5. Why EDs become crowded

A widely used model of ED crowding describes three parts: input, throughput and output. Input is the number and type of arrivals. Throughput is how fast the ED can assess, test and treat patients. Output is how fast patients can leave, and especially how fast admitted patients can

move to inpatient beds. When inpatient beds are full, admitted patients stay in ED beds, which is called boarding. Boarding is recognised as one of the main causes of ED crowding because it removes ED beds from use for new arrivals. Therefore a rise in waiting time with a fall in available ED beds usually points to an output problem, which can be solved by faster discharge and bed allocation upstairs, and not only by adding ED staff.

6. Flow tools used by the ED

Fast track is a separate area for ESI 4 and ESI 5 patients so that minor cases do not wait behind complex cases. Physician in triage means a doctor joins the triage nurse during busy periods to start orders early. Point-of-care tests are used for quick results. The ED observation unit holds patients who need a few hours of monitoring before a decision. The ED bed manager contact reports every 2 hours the number of patients waiting for inpatient beds.

7. Left without being seen

Patients who leave before seeing a doctor are recorded as left without being seen. The triage nurse tries to contact patients who left and who were ESI 2 or ESI 3. A rising left-without-being-seen rate above 2 percent is an indicator that waiting time is too long.

8. Special arrivals

Children and pregnant patients are directed to the Pediatrics and Obstetrics emergency entries and do not wait in the general ED queue. Ambulance arrivals are received by the ED team at the ambulance bay, and the ED must be told in advance by the ambulance crew if the patient is in a critical condition. In India, the national emergency number is 112, and ambulance services are available on 102 and 108 in many states.

9. Reading the ED dashboard

If arrivals are high but waiting time is normal, the ED is coping and should be watched. If waiting time is high and available ED beds are low, check boarding and inpatient bed availability. If waiting time is high but beds are available, check doctor and nurse availability, laboratory and radiology turnaround, and triage delays. If the left-without-being-seen rate rises, waiting is already affecting patients. The Overcrowding and Surge Capacity Plan lists the actions to take at each capacity level.